

Amendment No. 1 to SB2067

Bailey
Signature of Sponsor

AMEND Senate Bill No. 2067

House Bill No. 1930*

by deleting all language after the enacting clause and substituting:

SECTION 1. Tennessee Code Annotated, Section 56-7-2356, is amended by deleting the section and substituting:

(a) As used in this section:

(1) "Health benefit plan":

(A) Means:

(i) A hospital or medical expense policy;

(ii) A health, hospital, or medical service corporation contract;

(iii) A policy or agreement entered into by a health insurer or a health maintenance organization contract offered by an employer; and

(iv) A certificate issued under the policies, contracts, or plans described in subdivisions (a)(1)(A)(i)–(iii); and

(B) Does not include:

(i) Plans administered pursuant to the TennCare program or a successor medicaid program provided for in title 71, chapter 5;

(ii) Plans administered pursuant to the CoverKids Act of 2006, compiled in title 71, chapter 3, part 11;

(iii) Plans administered pursuant to the Access Tennessee Act of 2006, compiled in part 29 of this chapter;

(iv) Plans administered by the state, including:

(a) A plan managed by the health care finance and administration division of the department of finance and administration or a successor division or department; and

(b) The group insurance plans offered under title 8, chapter 27; and

(v) Limited benefit policies, including:

(a) Hospital confinement indemnity;

(b) Disability income;

(c) Accident only;

(d) Long-term care;

(e) Medicare supplement;

(f) Limited benefit health;

(g) Specified disease indemnity;

(h) Sickness or bodily injury or death by accident, or both; and

(i) Other limited benefit policies;

(vi) Coverage issued as a supplement to liability insurance;

(vii) Workers' compensation insurance;

(viii) Automobile medical payment insurance; or

(ix) Insurance under which benefits are payable with or without regard to fault and that is statutorily required to be contained in a liability insurance policy or equivalent self-insurance;

(2) "Health insurer" has the same meaning as defined in § 56-7-2355(a);

(3) "Managed health insurance issuer" has the same meaning as defined in § 56-32-128(a);

(4) "Material change":

(A) Means a significant reduction in the number of providers available as a participating provider in connection with a health benefit plan; and

(B) Includes:

(i) A reduction of ten percent (10%) or more of a specific type of provider in a geographic market;

(ii) The removal of a major health system that causes a network to be significantly different from the network when the beneficiary enrolled in the health benefit plan; or

(iii) A change that would cause the network to no longer satisfy the requirements of this section or the commissioner's rules for network adequacy;

(5) "Participating provider" means a provider that, under a contract with the health insurer or with its contractor or subcontractor, has agreed to provide one (1) or more healthcare services to enrollees of the health benefit plan with an expectation of receiving payment, other than coinsurance, copayments, or deductibles, directly or indirectly from the health insurer;

(6) "Physician" means a person licensed or permitted to practice medicine and surgery under title 63, chapter 6, or osteopathic medicine and surgery under title 63, chapter 9; and

(7) "Provider" means a physician, hospital, or other person licensed, accredited, or certified to perform specific healthcare services pursuant to title 63 or title 68.

(b) Each health insurer that offers a health benefit plan that has participating providers shall, for each health benefit plan:

(1) Prior to offering a health benefit plan, receive original approval from the department of commerce and insurance of the health benefit plan's network adequacy compliance with the network adequacy standards in this section;

(2) Maintain a network of participating providers compliant with network adequacy standards set forth in this section by monitoring compliance on an ongoing basis and engaging with physicians and healthcare providers for participating provider status;

(3) Maintain a directory of participating providers for enrollees that is available online and is updated no less frequently than weekly; and

(4) Report a material change from network adequacy standards to the department within fifteen (15) days of the date that the material change occurred, and:

(A) Act promptly to take a corrective action required to ensure that the network is compliant no later than ninety (90) days after the date the material change occurred; or

(B) Submit with the report of material change a request for a twelve-month waiver of a specific network adequacy standard because:

(i) There are no uncontracted licensed physicians or healthcare providers to meet the cited standard in the waiver request; or

(ii) The sole licensed physician or provider in the community to meet the cited standard in the waiver request refuses to engage in contract negotiations or refuses to agree to contract negotiation terms.

(c) The department:

(1) Shall, upon receiving a health insurer's health benefit plan network of participating providers for original approval under subdivision (b)(1), health insurer's report of a material change under subdivision (b)(4), or a complaint from an enrollee under subsection (g), perform an examination of the health benefit plan's network adequacy;

(2) Shall, upon receiving a health insurer's report of material change under subdivision (b)(4) or a complaint from an enrollee under subsection (g) of a suspected material change, notify the public on the department's website that the department is performing an examination into the health insurer's compliance with network adequacy standards for the specific health benefit plan; post on a national system for filing electronic rates and form filings, if the department participates in such system; and require the health insurer to provide a similar notice on the landing page for enrollees of the health benefit plan's website for the duration of the examination and for any period of a corrective action;

(3) Shall, in performing an examination of a health insurer's health benefit plan network of participating providers, require the health insurer to submit:

(A)

(i) A searchable and sortable database of network physicians and healthcare providers that are currently participating providers by national provider identifier, county, physician specialty, hospital privileges and credentials, type of healthcare provider licensure, as applicable, and which physicians or healthcare providers were or will be impacted by any material change;

(ii) Actuarial data of current and projected number of insureds by county;

(iii) Actuarial data of current and projected utilization of each participating provider type in the network;

(iv) A list of physicians and healthcare providers by group, if applicable, the health insurer unsuccessfully attempted to negotiate with for participating provider status, the dates of communication, and the name and contact information for the physician or healthcare provider; and

(v) Evidence from an appropriate licensing board that:

(a) There are no physicians or healthcare providers of the type of specialty or subspecialty in question available within the county; or

(b) In combination with the list of current participating providers, shows there are no uncontracted licensed physicians and healthcare providers to meet a specified network adequacy standard;

(4) Shall notify affected physicians and healthcare providers that they may be the subject of a network adequacy examination on behalf of the health insurer to meet network adequacy standards and provide the physicians and healthcare providers with an opportunity to submit evidence, including written testimony;

(5) Shall consider in an examination all written and oral testimony and evidence submitted by the health insurer and an enrollee pertinent to the health benefit plan's compliance with network adequacy standards under this section, including:

(A) The total number of physicians or healthcare providers in each participating provider specialty and subspecialty within the health benefit plan network available to contract with and whether the health

insurer made an effort to contract to meet the network adequacy standards of this section;

(B) The total number of facilities, and availability of pediatric, for-profit, nonprofit, tax-supported, and teaching facilities, within the county and service area and whether the health insurer made an effort to ensure accessibility and availability of covered benefits by participating providers within those in-network facilities;

(C) Population, density, and geographic information to determine the possibility of meeting reasonable time and distance, travel, and timely appointment requirements by participating physicians and healthcare providers to ensure accessibility and availability of covered benefits by a participating provider; and

(D) Availability of the types and specialty of participating healthcare providers to ensure the timely and convenient access to covered health benefits in the health benefit plan for all enrollees individually and collectively; and

(6) Shall not:

(A) Consider the availability of balance billing or surprise billing protections for an enrollee in determining whether a health benefit plan meets a network adequacy standard in this section; and

(B) Deem a health benefit plan's network inadequate solely because the network does not include a healthcare provider, specialist, or subspecialist who is the sole provider in the community, if the provider, specialist, or subspecialist refuses to contract with the health insurer's efforts to contract on terms and conditions substantially similar to healthcare providers, specialists, and subspecialists in similarly sized communities.

(d) The department:

(1) Shall complete a network adequacy examination and issue a ruling:

(A) Of denial or approval within thirty (30) days of receipt of a request for an original approval under subdivision (b)(1); or

(B) Of whether the network is or is not compliant with this section within sixty (60) days of receipt of a report of a material change under subdivision (b)(4) or receipt of a complaint under subsection (g);

(2) May issue a twelve-month waiver for a specific network adequacy standard under this section after the department confirms with appropriate licensing entities there is no uncontracted physician or healthcare provider in the county of a service area to meet a specific network adequacy standard or no healthcare provider, specialist, or subspecialist under subdivision (c)(6)(B) after the department confirms the healthcare provider's status as the sole provider in the community with the appropriate licensing agency, the provider's refusal to contract and the health insurer's attempt to meet the network adequacy standard;

(3) Shall, after receiving a report of a material change under subdivision (b)(4) or a complaint under subsection (g):

(A) Make a determination on whether the health insurer made a timely filing of a material change and waiver request under subdivision (b)(4);

(B) Make a determination of compliance for the health insurer's health benefit plan for reaching the network adequacy standard in this section;

(C) Make a determination on whether the health insurer made an effort to achieve a sufficient number of participating providers to meet network adequacy standards in this section;

(D) If the health benefit plan is noncompliant with a network standard, require:

(i) A modification to the health benefit plan network within sixty (60) days of the ruling; or

(ii) A corrective action plan for a health benefit plan to ensure access for enrollees;

(E) Assess a monetary penalty of not more than ten thousand dollars (\$10,000) for each claim submitted by a non-participating provider of a type or specialty in which the health insurer failed to meet network adequacy standards between when the material change occurred and when the health benefit plan met the network adequacy standard;

(F) Assess treble damages for monetary penalties under subdivision (d)(3)(E) if the health insurer failed to timely file the material change report under subdivision (b)(4); and

(G) For repetitive violations, refer the health insurer to the commissioner of commerce and insurance for an unfair trade practice as defined in § 56-8-104 in violation of § 56-8-103; and

(4) Shall post all penalties and corrective action plans under this section on the department's website in a conspicuous location for consumers and enrollees of health insurance.

(e) In an effort to ensure that enrollees have an adequate opportunity to select their choice of participating healthcare provider, including specialty healthcare providers and facilities, and their availability and accessibility of covered benefits to avoid delay of care and unanticipated out-of-network costs, health insurers offering a health benefit plan with participating providers shall meet network adequacy standards that include a full range of contracted physicians and healthcare providers as participating providers in

their network to provide covered healthcare services for adult and minor enrollees. Such standards must require for current and projected utilization:

(1) An adequate number of adult and pediatric primary care providers per capita of enrollees within not more than thirty (30) miles distance or thirty (30) minutes travel time at a reasonable speed;

(2) An adequate number of participating providers in sufficient numbers by adult and pediatric specialty and subspecialties to ensure choice, access, and quality of care for all covered benefits within a reasonable distance from the enrollee in a reasonable distance and time to travel;

(3) Enrollees to have the option of facilities that include, if available, pediatric, for-profit, nonprofit, and tax-supported institutions;

(4) Provision of necessary hospital services by requiring contracting with general, pediatric, specialty, and psychiatric hospitals on a participating provider basis within a reasonable distance from the enrollee;

(5) An adequate number of participating physicians with admitting privileges at participating provider hospitals necessary to make any necessary hospital admissions in a reasonable time for routine, acute, and emergency issues;

(6) An adequate number of acute care hospitals within a reasonable distance from enrollees; and

(7) For each participating provider hospital, ambulatory surgical treatment center, and facility, participating physicians and healthcare providers are provided who are credentialed within the facility for the specialties of anesthesiology, emergency care, neonatology, pathology, radiology, radiation oncology, and both general and specialty surgery to ensure access and availability for enrollees of covered services available at each facility.

(f)

(1) In any case where a managed health insurance issuer has no participating providers to provide a covered benefit, the managed health insurance issuer shall arrange for a referral to a provider with the necessary expertise and ensure that the covered person obtains the covered benefit at no greater cost to the covered person than if the benefit were obtained from a network provider.

(2) In determining whether a managed health insurance issuer has complied with this section, consideration must be given to the relative availability of healthcare providers, specialists, and subspecialists in the service area under consideration. Relative availability includes the acceptance by the healthcare providers, specialists, or subspecialists of the terms, conditions, and fees offered under the contract or plan. A network must not be deemed inadequate solely because the network does not include a provider, specialist, or subspecialist who is the sole provider in the community, if the provider, specialist, or subspecialist refuses to contract with the managed health insurance issuer on terms and conditions substantially similar to providers, specialists, or subspecialists in contiguous communities.

(3) Healthcare providers who participate in a managed health insurance issuer's plan shall provide timely appointments to patients and shall see the patients on a timely basis, after arrival for an appointment. A managed health insurance issuer may include in its contracts with healthcare providers provisions that establish the meaning of timeliness.

(4) Providers who do not participate in a managed health insurance issuer's plan but seek reimbursement through the point of service option mandated in § 56-32-128 shall provide appointments on a timely basis and, upon arrival for appointments, the provider shall see the patient on a timely basis; provided, that timeliness must be consistent with usual and customary standards

for the community in which the provider is located. Frequent and repetitive failure to comply with this subdivision (f)(4) may be cause for the managed health insurance issuer to withhold reimbursement from the non-participating provider.

(g)

(1) An enrollee of a health benefit plan, a healthcare provider, or facility may file a complaint against a health benefit plan, which must prompt an examination of network adequacy standards under subsection (c). A complainant may file a complaint electronically on the department's website or in writing to the department's insurance division.

(2) The complaint must:

(A) Name the health insurance issuer and the specific health benefit plan or plans for which the complainant requests an examination;

(B) Name the network adequacy standard for which the complainant questions the compliance of the health benefit plan with this section;

(C) Include the name, address, telephone number, and electronic mail contact information for the complainant;

(D) Include a summary of the reason the complainant questions the compliance of the health benefit plan; and

(E) Include any supporting documentation.

(3) The department shall acknowledge receipt of the complaint, provide a complainant the date by which the examination and ruling will be completed and, in addition to public notices in this section, notify the complainant of the ruling, any required corrective action, and penalties assessed against the health insurer.

(4) The department may request information, as needed, for its examination, from the complainant or the health insurer. Any information that a

party deems proprietary, or trade secrets, is not a public record subject to disclosure and is exempt from title 10, chapter 7.

(h)

(1) By January 15, 2027, and on or before January 15 of each subsequent year, the department shall provide a report to the speakers of the senate and house of representatives and chairs of the health and welfare committee of the senate and the committee of the house of representatives having jurisdiction over health insurance on the health insurers' compliance with this section. The department shall publish a copy of the report on its website. The report shall provide information on the compliance with this section by health insurers in a way that shows insight on the trends of material change reports, complaints, and outcomes of examinations over time and within the previous year in the aggregate and without identifying individual health insurers, healthcare providers, or complainants.

(2) Health insurers shall submit an annual report to the department, no later than October 1, 2026, and October 1 of each subsequent year, for each of their health benefit plans that includes:

(A) A copy of their searchable database under subdivision (c)(3)(A) that was the subject of their most recent approval by the department under an examination for reasons in subdivision (b)(1) or (4) or subsection (g);

(B) Any changes to the list of providers since that last examination; and

(C) A list of any original requests for approval, material change reports, or complaints filed against a health benefit plan in the last twelve (12) months and the outcomes of the subsequent examinations and any corrective actions.

SECTION 2. If any provision of this act or its application to any person or circumstance is held invalid, then the invalidity does not affect other provisions or applications of the act that can be given effect without the invalid provision or application, and to that end, the provisions of this act are severable.

SECTION 3. This act takes effect upon becoming a law, the public welfare requiring it.